AMARA HEALTH · AFRICA PROGRAMME · INVESTOR BRIEF

Amara First Contact: right care from the *first door.*

Hai, Kilimanjaro, Tanzania
November 2026

An offline AI app that helps accredited drug shops (ADDOs) and community health workers in rural Tanzania triage and refer mothers and children to the right clinic, on a low-cost Android phone with no internet.

PROBLEM

4.4 / 10,000

Health workers per 10,000 people in Tanzania; WHO recommends 44.5.

FIRST STOP

31%

Of people go to a drug shop first when ill, more than to a public facility.

MISSED

60.7% vs 1.6%

Severe fevers diagnosed as malaria vs. those that were malaria (northern Tanzania).

SOLUTION

1 She speaks Swahili

A 660 KB intent model on the phone maps her words to a fixed list of symptoms and exposures. Below 40% confidence it says "Not sure".

2 Rules set urgency

Sourced Tanzanian and WHO guideline rules decide: Go now, refer today, or within 3 days. The dispenser confirms every flag.

3 She gets an SMS

Clinic, time and code, no health details. Her note and draft ICD-10 codes reach the clinician first; anonymous counts flag outbreaks.

BUSINESS MODEL

Free for mothers. Paid by health programmes. NGO and donor programmes first (setup + per site), then per completed referral, district licences and insurers.

$24k

A year per district: 100 sites × 20 referrals/month × $1. Illustrative.

$1 vs ~$170

Referral vs. one m-mama emergency transport (Munishi 2023).

MARKET

~$4M /yr

Tanzania potential: ~170 district councils × $24k. Illustrative.

2.4M

Births a year in Tanzania; 9,000+ accredited drug shops.

Beachhead: Hai District → Kilimanjaro → Tanzania → Kenya, Uganda, Rwanda.

WHY WE WIN

Fully offline AI

Triage needs no data or signal; only the SMS needs a network.

WHY WE WIN

At the drug shop counter

Catches mothers where they go first, before an emergency. Complements m-mama and Afya-Tek.

WHY WE WIN

Swahili built for clinics

An intent model for how mothers describe symptoms and exposures, with sourced rules on top.

TRACTION & ROADMAP

Oct 2026 — MVP
Built at the World Bank × Hack-Nation hackathon

Q1 2027 — Validate
ADDO interviews, clinical review, NIMR ethics

Q2–Q3 2027 — Hai pilot
Drug shops and linked facilities

2028 — Scale
District contracts, then region

THE ASK

- Pilot funding: phones, training, SMS, a facilitator in Hai
- Clinical reviewers and an implementing partner in Tanzania
- Introductions to district and regional health teams

Governance: non-diagnostic triage; clinicians decide. We will confirm TMDA's classification of decision-support software before the pilot.

TEAM

Juanita Ndyamukama · Co-founder · Johns Hopkins Bloomberg School of Public Health · Tanzanian

Erica Stephens · Co-founder · Johns Hopkins Bloomberg School of Public Health

SEE IT

Website: ericastephens.github.io/amara-first-contact/story

Live demo: ericastephens.github.io/amara-first-contact

LinkedIn: juanita-ndyamukama · ericasstephens

Sources: WHO GHO 2024; Embrey et al., PLOS ONE 2016; Crump et al., PLOS NTDs; UN WPP 2024; Munishi et al., PLOS Glob Public Health 2023; ICLD 2025, CLGF (councils). Figures marked illustrative are hypotheses to test in the pilot.

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